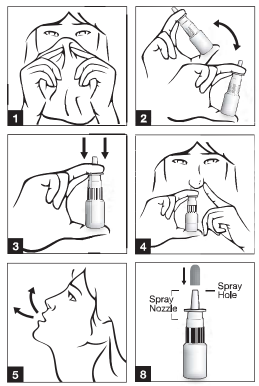
Acute Rhinosinusitis: Adults (including geriatric patients) and adolescents 12 years of age or older: The usual recommended dose for acute rhinosinusitis is two sprays (50 micrograms/spray) in each nostril twice daily (total daily dose of 400 micrograms). If no improvement is seen after 15 days of twice daily administration, alternative therapies should be considered. If symptoms worsen during treatment, the patient should be advised to consult their physician.

DRUG INTERACTIONS: Not known

ADVERSE EFFECTS: Most severe side effects include allergic reaction: difficulty in breathing; swelling of the face, lips, tongue, or throat. Serious side effects include severe and ongoing nose bleed; sores in the nose; wheezing, troubled breathing; body aches, or flu symptoms. Less serious side effects may include headache; sore throat, cough; muscle or joint pain.

DIRECTION FOR USE:

1. Blow the nose gently to clear nostrils.
2. Shake the bottle gently. Remove the cap.
3. For first time use, hold the bottle upright and fill the pump by pressing the nozzle downwards and releasing it 5 times, until a fine spray is produced.
4. Close one nostril with finger. Hold the bottle upright and insert the nozzle into the nostril as far as is comfortable. Whilst breathing in gently through the nose with the mouth closed, press the bottle upwards once to deliver one spray.
5. Breathe out through the mouth.
6. Repeat steps 4 and 5 for the other nostril.
7. For patients aged 12 years and older, steps 4, 5 and 6 should be repeated once more to achieve the recommended dosage of 2 sprays into each nostril.
8. To keep the spray nozzle clean, wipe it carefully with a clean tissue after each use and replace back the cap.



If the nasal spray has not been used for 2 weeks or more, it needs to be sprayed once into the air before use. The nozzle should be pointed away while doing this. Always shake the bottle gently before use.

If the spray does not work and it may be blocked, clean it as follows. (***See Directions for Cleaning***). NEVER try to unblock it or enlarge the tiny spray hole with a pin or other sharp object because this will destroy the spray mechanism.

The nasal spray should be cleaned at least once a week or more often if it gets blocked.

Directions for Cleaning:
1. Remove the cap and pull off the spray nozzle only.
2. Soak the dust-cap and spray nozzle in warm water for a few minutes, and then rinse under cold running tap water.
3. Shake or tap off the excess water and allow to air-dry.
4. Re-fit the spray nozzles.
5. Prime the unit as necessary until a fine mist is produced and use as normal.

CONTRAINDICATIONS: Hypersensitivity to any ingredients of ELONIDE Nasal Spray 50 mcg/dose.

PRECAUTIONS/WARNING: ELONIDE Nasal Spray should be used with caution, if at all, with active or quiescent tuberculous infections of the respiratory tract, or in untreated fungal, bacterial, systemic viral infections or ocular herpes simplex.

Following 12 months of treatment with ELONIDE Nasal Spray there was no evidence of atrophy of the nasal mucosa; also, mometasone furoate tended to reverse the nasal mucosa closer to a normal histologic phenotype. As with any long-term treatment, patients using ELONIDE Nasal Spray over several months or longer should be examined periodically for possible changes in the nasal mucosa. If localised fungal infection of the nose or pharynx develops, discontinuance of ELONIDE therapy or appropriate treatment may be required. Persistence of nasopharyngeal irritation may be an indication for discontinuing ELONIDE Nasal Spray.

Although ELONIDE Nasal Spray will control the nasal symptoms in most patients, the concomitant use of appropriate additional therapy may provide additional relief of the symptoms, particularly ocular symptoms.

There is no evidence of hypothalamic-pituitary adrenal (HPA) axis suppression following prolonged treatment with ELONIDE Nasal Spray. However, patients who are transferred from long-term administration of systemically active corticosteroids to ELONIDE Nasal Spray require careful attention. Systemic corticosteroid withdrawal in such patients may result in adrenal insufficiency for a number of months until recovery of HPA axis function. If these patients exhibit signs and symptoms of adrenal insufficiency, systemic corticosteroid administration should be resumed and other modes of therapy and appropriate measures instituted.

During transfer from systemic corticosteroids to ELONIDE Nasal Spray some patients may experience symptoms of withdrawal from systemically active corticosteroids (e.g. joint and/or muscular pain, lassitude, and depression initially) despite relief from nasal symptoms and will require encouragement to continue ELONIDE Nasal Spray therapy. Such transfer may also unmask pre-existing allergic conditions, such as allergic conjunctivitis and eczema, previously suppressed by systemic corticosteroid therapy.

The safety and efficacy of ELONIDE Nasal Spray has not been studied for use in the treatment of unilateral polyps, polyps associated with cystic fibrosis, or polyps that completely obstruct the nasal cavities.

Patients receiving corticosteroids who are potentially immunosuppressed should be warned of the risk of exposure to certain infections (e.g. chickenpox, measles) and of the importance of obtaining medical advice if such exposure occurs.

Following the use of intranasal corticosteroids, instances of nasal septum perforation or increased intraocular pressure have been reported very rarely.

Systemic effects of nasal corticosteroids may occur, particularly at high doses prescribed for prolonged periods. Growth retardation has been reported in children receiving nasal corticosteroids at licensed doses.

It is recommended that the height of children receiving prolonged treatment with nasal corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of nasal corticosteroid if possible, to the lowest dose at which effective control of symptoms is maintained. In addition, consideration should be given to referring the patients to a paediatric specialist.

Treatment with higher than recommended doses may result in clinically significant adrenal suppression. If there is evidence for higher than recommended doses being used, then additional systemic corticosteroid cover should be considered during periods of stress or elective surgery.